7.4 Renal and Ureteric Calculi*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The pain comes in waves from my back to my groin." • Rates pain 10/10 **Objective** • Restless; cannot find a comfortable position • Hematuria • Nausea	**ACTUAL** Acute pain related to ureteric obstruction and spasm by a calculus as evidenced by severe colicky flank pain.	• Pain at or below 3/10 within an hour of analgesia.	• Give an NSAID or opioid as prescribed and reassess. • Encourage 2–3 L of fluid a day unless contraindicated. • Strain all urine and keep any stone for analysis. • Encourage walking to help the stone pass.	• Goal reached when pain is controlled.
Subjective • "I pass small amounts of urine often." **Objective** • Frequency • Hematuria	**ACTUAL** Impaired urinary elimination related to ureteric obstruction as evidenced by frequency and hematuria.	• Voids normally once the stone has passed or been removed.	• Keep an intake and output chart. • Report anuria at once. • Prepare for lithotripsy or ureteroscopy as ordered.	• Met when voiding is normal.
Subjective • "I drink very little water at work." **Objective** • Has had a stone before • Works in heat and drinks little	**RISK** Risk for infection as evidenced by risk factors: urinary stasis proximal to an obstructing calculus.	• Knows how to prevent infection and recurrence, and when to seek urgent care.	• Teach him to drink enough to pass 2–2.5 L of urine a day. • Give dietary advice based on the stone type. • Tell him to seek urgent care for fever with flank pain.	• Achieved when prevention and warning signs are explained back.

* **References for this care plan:** [14], [32], [35], [37]. Full entries are listed in the References section of the CPMN book.